A self-guided approach to recovery from **OCD**

Self-therapy Guidebook:

INTRUSIVE THOUGHTS + OBSESSIVE COMPULSIVE DISORDER (OCD)

FIRST EDITION

Greylight Psychology

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PLEASE NOTE:

The material within this resource is not intended as a replacement for one-on-one sessions with a qualified mental health professional.

If you are struggling with genuine mental health challenges, it is advised that you are properly assessed and provided with a relevant, evidence-based treatment plan by your attending practitioner.

If you are at risk of self-harm or suicide, please contact the emergency supports local to you, or present to your local emergency department.

Firstly, welcome!

If you're reading this, you've likely decided to make a significant change in your life, and more to the point, you've chosen to experiment with applying the techniques yourself. This is by no means an easy task, and you will likely encounter additional and unique challenges compared to those working alongside a professional, but you'll also encounter unique and exciting opportunities on this guided journey towards recovery and a greater degree of purpose and happiness.

It must be said that self-therapy is not intended to be a complete replacement of traditional face-to-face therapy with a qualified and experienced professional. Instead, the goal of this ebook is to provide you with an opportunity to experiment with learning and applying the skills taught in evidence-based settings to your own unique situation, and to provide some relief to our overburdened mental health system. More importantly, it seeks to provide relief to you, who might be struggling to find any psychologists local to you with reasonable enough availability.

A few notes on recovery

In psychology, recovery is a dynamic word that encapsulates the ebb and flow of progress through therapy. It is a 'moving' word, in that there are periods of growth and periods of relapse, but regardless, the journey towards a significant remission continues. Is there a point at which someone is considered to have 'recovered'? Possibly, but psychologists generally don't view recovery as a singular point or a threshold that is permanently crossed, never to be stepped back behind again.

As with most issues in life, problems we thought were dealt with can resurface, and the disappointment or shame after reencountering an issue we thought we 'cured' can drag us down far quicker and deeper than necessary. This is not to say that we must live in fear of past challenges reemerging when we're tired or vulnerable, but that we should remain prepared and always within arm's reach of our tools and management strategies.

Given this, recovery is about 'two steps forward, and one step back', and it's vital that we make room for the human element on this journey. We're going to make mistakes, and that's okay. We're going to rebel against what we know we should do, and that's okay too. Ultimately, recovery is about getting up after each fall, dusting ourselves off and soldiering forwards towards what we know we deserve, Human beings, and our brains more specifically, are wholly resistant to change, but the task is not impossible – far from it. With enough will power, discipline and grit, we can start to see a healthier, more idealised quality of life take shape, and this is one of the most beautiful experiences life on this planet has to offer.

A typical course of therapy

A standard course of face-to-face therapy can last anywhere between 10 to 20 sessions, depending on the pace of treatment, client characteristics and the severity of symptoms. The structure of this ebook is divided into Modules, with each Module accounting for approximately 2 to 3 sessions in conventional therapy. While you don't have to space them out fortnightly, you are welcome to do so. You are also free to move as quickly, or as deliberately, as you see fit, and if you have some experience with therapy for generalised anxiety, you may wish to skip straight to the content applicable to your needs and goals.

What is self-therapy?

As the name suggests, self-therapy is about you guiding yourself through the recovery journey. It takes the core principles, psychoeducation and strategies from traditional in-room therapy, and places them at your disposal, while guiding you steadily so as to avoid moving too quickly.

It differs from self-help in that the information in this ebook is rigourously validated and drawn from a large variety of scientific studies, and seeks to emulate the pace and 'feel' of a guided therapeutic course. It is practical, but also clinical, and will focus largely on what has demonstrated to work at reducing symptoms as opposed to what makes you feel 'warm and fuzzy' in the short-term. This is not to say that it's 'superior' to self-help – it's just a different approach, and each individual will likely develop a preference for one over the other.

Contact us!

If you have any questions regarding the material, you can always contact us at our instagram page via direct message: ***@greylight_psychology*** or via our email address: ***enquiries@greylightpsychology.com.au***

We wish you the very best, and truly hope that you find great value in the pages to follow.

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Module 1.1 - Psychoeducation

OBSESSIVE COMPULSIVE DISORDER

Overview of OCD

What's the first thing that comes to mind when you hear the letters O-C-D? Is it someone scrubbing their bodies for countless hours attempting to rid themselves of harmful contaminants that could infect a loved one? Perhaps it's a child who needs to have all their toys organized neatly from biggest to smallest, and a tantrum ensues if one is moved out of place? Maybe it's an adolescent that must step in the middle of doorways to prevent some terrible fate befalling a parent?

While the above are all serviceable examples of what we might call "classic" presentations, in reality, the disorder is considerably more varied and insidious. For some individuals with OCD, there isn't an obvious behaviour or 'tell' to bring their pain to light, and instead, they're left to struggle within a seemingly endless mental prison. For example, some individuals with OCD will ruminate tirelessly on the question of whether they wish to harm their family and closest friends, and experience extreme anxiety at the prospect that these thoughts may be indicative of a genuine desire. This sub variant of OCD, known as **Harm-OCD**, is one of many examples of a lesser known type of the disorder called **Pure-Obsessional OCD**, or **Pure-O**. Pure-O is mechanically identically to classic OCD presentations, although the compulsions used to counteract anxiety (see below) are almost entirely mental.

Generally speaking, OCD is characterised by two broad features, with the first, **obsessions**, accounting for unwanted experiences intruding

into consciousness that may be experienced as urges, images or thoughts, and the second, **compulsions**, accounting for later automatic behaviours that are used to reduce the resulting emotional discomfort, which is typically experienced as anxiety.

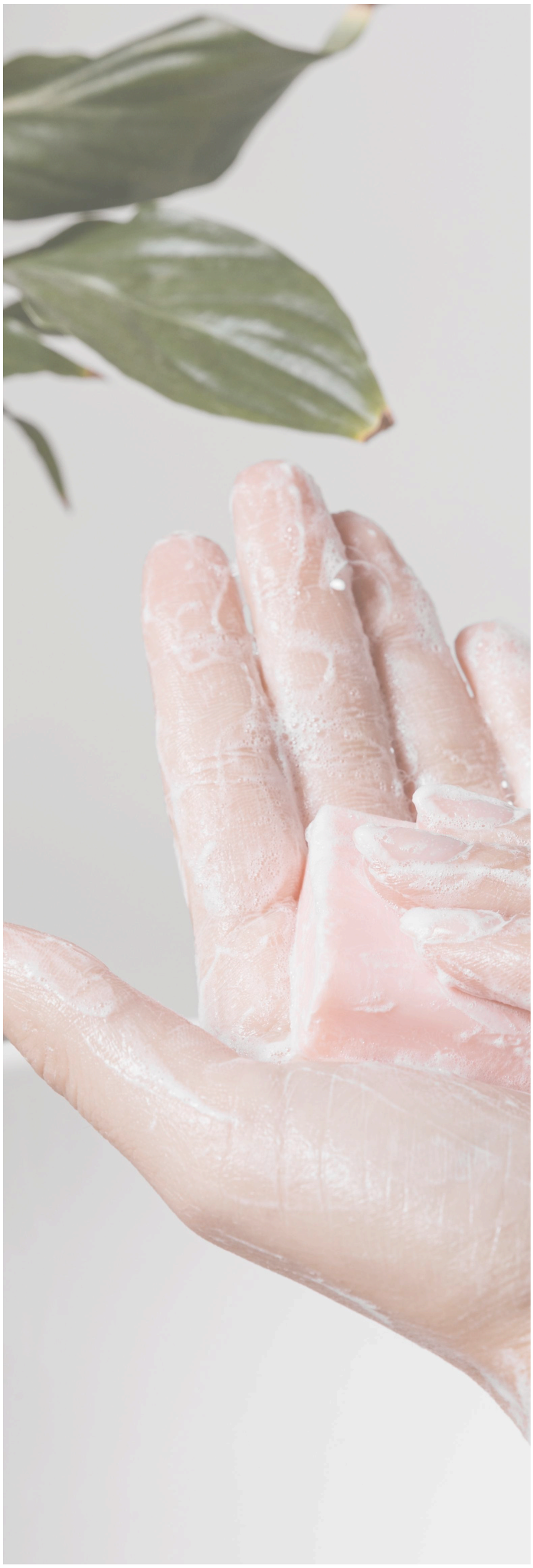
The objective of treatment for OCD is to weaken the fearful associations held by an individual through self-initiated contact with the situations or circumstances that trigger emotional distress, while discouraging the use of unhealthy behavioural responses that keep the cycle of obsession in motion. Referred to as **exposure**, this self-initiated contact is most effective when designed to trigger a tolerable amount of distress before moving onto challenges that bring about a greater degree of anxiety. When exposures are organised and tackled from least anxiety-provoking to most anxiety-provoking, individuals are more likely to experience a gradual reduction of their anxiety in relation to intrusive thoughts.

What causes OCD?

Like all diagnoses, figuring out how OCD manifests is complex, and there are usually a number of factors contributing to its development. Researchers have indicated the high probability of genetics playing a key role, with the identification of several potentially relevant genes, but studies have also concluded that more research is required. Environmental factors can also influence the likelihood of OCD symptom development as early experiences with anxiety and the behaviours used to successfully reduce distress may lay the foundation for future OCD-type patterns.

Parenting and parental modelling are also thought to influence symptoms, in that children of parents with symptoms or symptom-adjacent tendencies may adopt traits or characteristics such as anxiety-sensitivity and anxiety-avoidance. Moreover, fixations on orderliness and rigid procedures, even in the absence of sense or logic, can be passed on in this way also.

Temperament, which refers to an individual's personality, general mood-state, or characterological qualities, can also contribute to the development of OCD. Individuals with trait shyness, emotionality, and behavioural inhibition tend to exhibit higher degrees of OCD symptoms.



The neurobiology of OCD

Neuroimaging studies of the brain have implicated the ***cortico-striato-thalamo-cortical circuit*** (**Figure 2.**) in the maintenance of OCD symptoms. The prevailing understanding amongst researchers is that this circuit, which has a ***direct (excitatory)*** and ***indirect (inhibitory) pathway*** (**Figure 1.**), is compromised in OCD brains as compared to healthy 'regular' brains. In the case of an OCD brain, the ***excitatory pathway*** has a lower "threshold" of activation, meaning that it may be more likely to register danger in the form of contamination or harm, even if no such danger is apparent.

This activation may then lead to obsessional fixation on the material emerging into consciousness as a result of this malfunctioning circuitry. Moreover, this ***excitatory pathway*** appears to operate as a positive feedback loop, meaning that obsessions or signals of danger increase in magnitude over time. It also appears to be the case that for OCD brains the ***inhibitory pathway***, which reduces signals of danger, is less effective, which may explain how and why intrusive thoughts are generated.

Figure 1. The excitatory and inhibitory pathways of the frontal-subcortical circuits

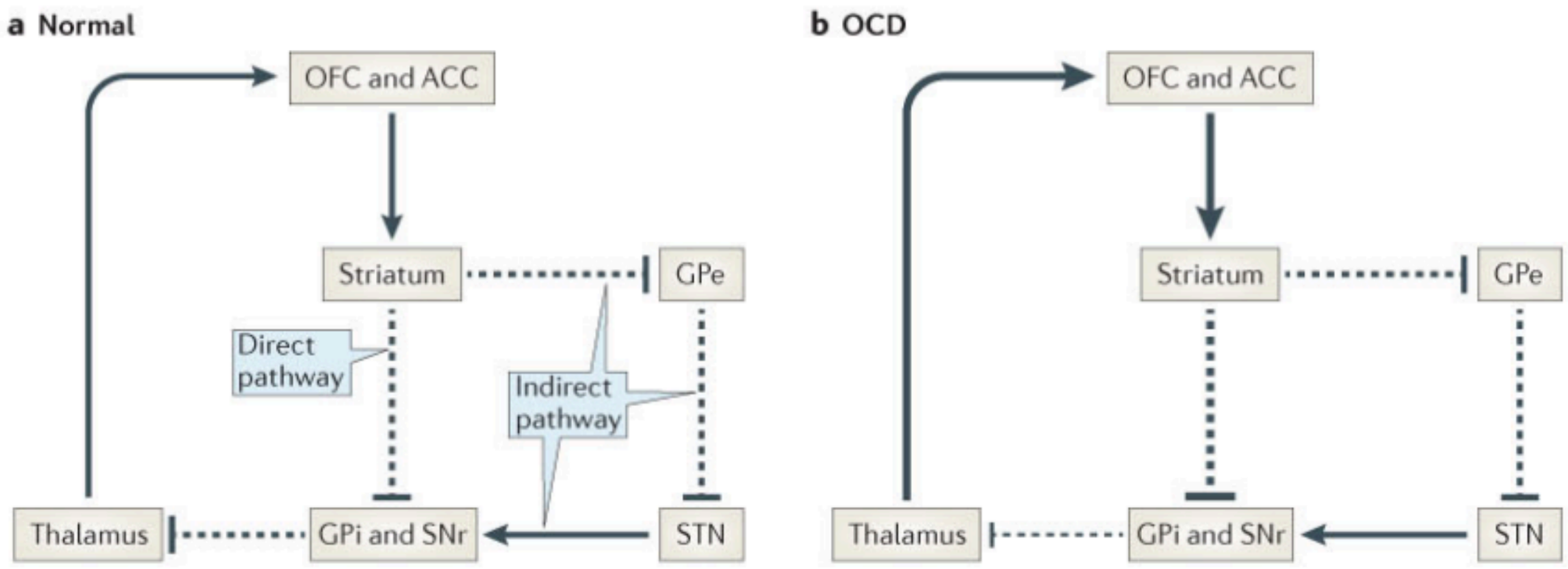
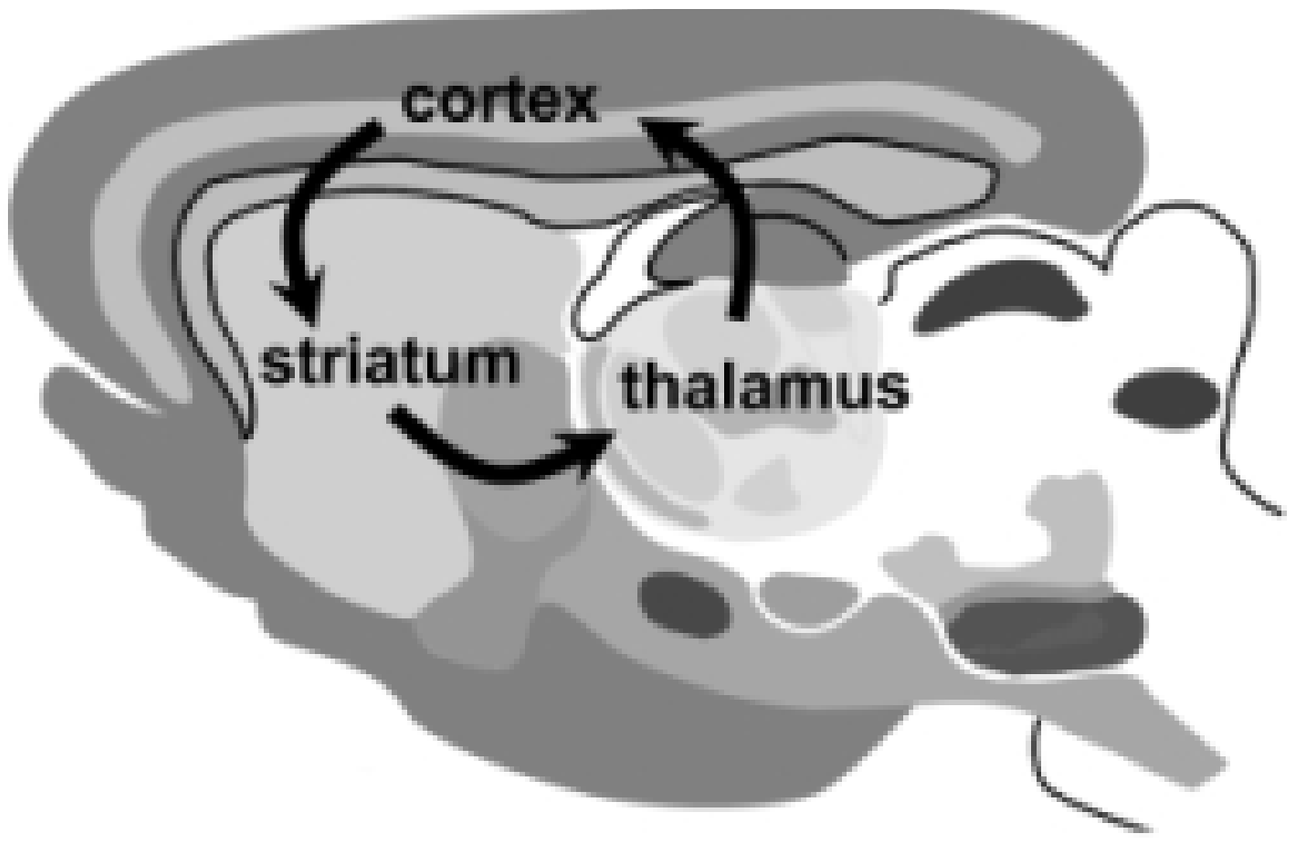


Figure 2. The cortico-striato-thalamo-cortical circuit



The 'chemical imbalance' theory

Dogmatic thinking, which is the rigid adherence to an ideal or position despite evidence to the contrary, is undoubtedly dangerous. Proponents of the idea that mental health can simply be reduced to an 'imbalance' to one or more neurotransmitters are likely oversimplifying the problem , and may (**read: definitely do!**) have vested interests in doing so (cha-ching!). While the chemical (monoamine) hypothesis has come under fire recently with the publishing of a review concluding there to be no link between depression and serotonin levels (University College of London, 2022), I believe it is equally as irresponsible to conclude that neurotransmitters have ZERO association with mental health.

So, what is a **neurotransmitter** exactly? Neurotransmitters are just that, *transmitters* of information, and they carry their messages via chemical signaling. There are varied perspectives on which are the 'core' neurotransmitters within our body, but a starting list might include **serotonin**, **dopamine**, **acetylcholine**, and **glutamate/GABA**.

The brain is the most complex organ in our body, and as much as we try to avoid this fact, we simply don't have it all figured out just yet. Take the neurotransmitter **serotonin** for instance. 80 to 90% of this monoamine is produced in our gut, and it has been implicated in a variety of physiological systems, including appetite, sleep, anxiety and memory.

Here's where things get complex..

Our brains come preloaded with a number of different serotonin receptor types, each serving a unique function from the rest. For example, the 5HT1A receptor, which is distributed throughout the raphe nuclei and the hippocampus, is associated with anxiety, sleep regulation and feeding, whereas the 5HT2A receptor located within the platelets and cerebral cortex is associated with the excitation (activation) of cells and muscle contractions.

Dopamine is no different. This neurotransmitter is primarily associated with motivation, learning, the anticipation of reward, attention, and organ regulation, and like serotonin, our brains contain a myriad of dopamine receptors that serve differing functions. Take the dopamine receptor D1 located in a number of brain structures. Its function is to coordinate attention, learning, sleep, impulse control and memory, whereas D2 receptors, for example, are associated with many of same functions with the addition of reproductive behaviour.

As you can see, there's a lot more to the brain, its chemical messengers and mental health than a mere imbalance.

Neurotransmitters and OCD

Emerging research suggests that abnormalities in neurotransmitter systems play a significant role in the etiology and manifestation of OCD symptoms. These include:

Serotonin (5-HT): Serotonin is perhaps the most extensively studied neurotransmitter in the context of OCD. Evidence suggests that dysregulation of the serotonin system, particularly reduced levels of serotonin, is associated with the pathophysiology of OCD (Albert et al., 2020). Selective serotonin reuptake inhibitors (SSRIs), which increase serotonin levels in the brain, are commonly used as first-line pharmacological treatments for OCD, further supporting the serotonin hypothesis of OCD.

Glutamate: Glutamate is the primary excitatory neurotransmitter in the brain and has been implicated in the neurobiology of OCD. Dysfunctions in glutamatergic neurotransmission, specifically abnormalities in glutamate levels or receptor functioning, have been observed in individuals with OCD (Fineberg et al., 2020). Emerging research suggests that targeting the glutamatergic system may offer novel therapeutic avenues for the treatment of OCD.

Dopamine: While traditionally associated with disorders like schizophrenia, dopamine dysregulation has also been implicated in OCD. Dysfunctions in the dopamine system, particularly alterations in dopamine receptor sensitivity or availability, may contribute to

certain OCD symptoms, such as reward-related behaviors and the experience of pleasure or satisfaction (Denys et al., 2019). However, the precise role of dopamine in OCD pathophysiology remains an area of ongoing investigation.

Nutritional factors: B-vitamins and OCD

B-vitamins, a group of water-soluble vitamins that play essential roles in cellular metabolism and neurological function, have garnered attention in recent years for their potential implications in mental health disorders, including Obsessive-Compulsive Disorder (OCD). Studies suggest that deficiencies in certain B-vitamins may contribute to alterations in neurotransmitter function and neuronal pathways implicated in OCD symptomatology.

Vitamin B6 (Pyridoxine): Vitamin B6 is involved in the synthesis of neurotransmitters such as serotonin, dopamine, and gamma-aminobutyric acid (GABA), which play critical roles in mood regulation and anxiety modulation, both of which are central to OCD pathology (Fernstrom, 2012). Research indicates that inadequate levels of vitamin B6 may disrupt neurotransmitter balance, potentially exacerbating symptoms of OCD (Sarris et al., 2012).

Vitamin B12 (Cobalamin): Vitamin B12 is essential for the maintenance of neuronal integrity and the synthesis of myelin, the protective sheath surrounding nerve fibers. Deficiencies in vitamin B12 have been linked to neurological symptoms such as cognitive impairment and mood disturbances, which may exacerbate OCD symptoms (Hutto, 2008). Furthermore, vitamin B12 plays a role in the methylation cycle, which regulates neurotransmitter metabolism, suggesting a potential mechanism by which its deficiency could impact OCD pathology (Bottiglieri et al., 2000).

Folic Acid (Vitamin B9): Folic acid is crucial for DNA synthesis and methylation processes involved in neurotransmitter production and regulation. Research suggests that deficiencies in folic acid may compromise serotonin synthesis and function, contributing to mood dysregulation and anxiety, which are prominent features of OCD (Young, 2007). Moreover, folic acid deficiency has been associated with elevated levels of homocysteine, an amino acid linked to vascular inflammation and oxidative stress, both of which may exacerbate OCD symptoms (Coppen & Bolander-Gouaille, 2005

To assess the status of your b-vitamins and overall health, be sure to contact your general practitioner to organise a blood pathology test.

DISCLAIMER: Speak with a qualified medical and/or dietary professional before considering supplementing with any products, or implementing drastic changes in diet.

MTHFR gene mutations

Prepare yourself for a mouthful!

Methylenetetrahydrofolate reductase (MTHFR) is a crucial enzyme involved in the methylation cycle, which regulates various biochemical processes, including neurotransmitter synthesis and DNA methylation. Genetic variations in the MTHFR gene, particularly the C677T and A1298C polymorphisms, have been associated with reduced enzyme activity and impaired folate metabolism, potentially contributing to alterations in neurotransmitter function and increased susceptibility to mental health disorders, including OCD.

Several studies have investigated the relationship between MTHFR gene mutations and OCD, with some suggesting a potential link between MTHFR polymorphisms and increased susceptibility to OCD symptoms. For example, a meta-analysis by Liu et al. (2018) found that individuals with the MTHFR C677T polymorphism may have a higher risk of developing OCD, particularly in certain ethnic populations.

The MTHFR enzyme plays a critical role in folate metabolism, and deficiencies in folate have been implicated in the pathophysiology of OCD. Folate is essential for neurotransmitter

synthesis and DNA methylation, processes thatare dysregulated in individuals with OCD. Therefore, genetic variations in the MTHFR gene that compromise folate metabolism may exacerbate OCD symptoms by disrupting neurotransmitter balance and altering gene expression patterns in the brain.

While the precise mechanisms underlying the relationship between MTHFR gene mutations and OCD remain to be fully understood, emerging evidence suggests that genetic factors may contribute to individual differences in susceptibility to OCD and response to treatment. Further research is needed to better understand the role of MTHFR gene polymorphisms in OCD pathology and to explore the potential implications for personalized treatment approaches.

You can assess your gene profile via genetic testing, although there is some controversy as to the accuracy of these tests. Your millage may vary.

DSM-5 CRITERIA FOR OBSESSIVE COMPULSIVE DISORDER

A. Presence of obsessions, compulsions, or both:

Obsessions are defined by (1) and (2):

1. Recurrent and persistent thoughts, urges, or images that are experienced, at some time during the disturbance, as intrusive and unwanted, and that in most individuals cause marked anxiety or distress.
2. The individual attempts to ignore or suppress such thoughts, urges, or images, or to neutralize them with some other thought or action (i.e., by performing a compulsion).

Compulsions are defined by (1) and (2):

1. Repetitive behaviors (e.g., hand washing, ordering, checking) or mental acts (e.g., praying, counting, repeating words silently) that the individual feels driven to perform in response to an obsession or according to rules that must be applied rigidly.
2. The behaviors or mental acts are aimed at preventing or reducing anxiety or distress, or preventing some dreaded event or situation; however, these behaviors or mental acts are not connected in a realistic way with what they are designed to neutralize or prevent, or are clearly excessive.

B. The obsessions or compulsions are time-consuming (e.g., take more than 1 hour per day) or cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

C. The obsessive-compulsive symptoms are not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

D. The disturbance is not better explained by the symptoms of another mental disorder.

DISCLAIMER: Diagnosis is beyond the scope of this ebook, and if you want to be properly assessed by a qualified professional, it is recommended that you book in an initial appointment with your local or family doctor. The DSM-5 criteria are included for reference, and should not be used as a means of self-diagnosis.

Prevalence

Obsessive-Compulsive Disorder (OCD) affects approximately 2% of the Australian population each year, and over the course of their lives, nearly 3% of Australians will experience symptoms reflective of a full clinical diagnosis. In the USA, approximately 1% of adolescents will experience lifetime prevalence, and slightly more than half of individuals meeting the criteria for diagnosis are female. Onset of symptoms typically occurs during early adolescence or young adulthood, with males often accruing an earlier age of onset than females. Obsessive-compulsive disorder follows a chronic ‘waxing and waning’ course, with symptom development often occurring gradually, although sudden onset of acute symptoms has also been observed.

PRESENTATIONS OF OCD

Sub-type	Description	Common Compulsions
Sexual Orientation OCD (SO-OCD)	Obsessive, disproportionate, and unfounded fears that an individual's sexual identity is different, or will change, from their prior established identity. For example, a lifelong heterosexual develops intense and intrusive thoughts that they are gay, despite no sexual attraction to same-sex individuals.	**Checking:** body scans in the genital area to 'test' for arousal when looking at same-sex individuals. **Reassurance seeking:** repeatedly asking friends, family, and spouses for reassurance that there is no change with respect to sexual orientation. **Avoidance:** any situations where same-sex individuals may be present.
Pedophile OCD (P-OCD)	Obsessive, disproportionate, and unfounded fears that an individual may be sexually attracted to children, or have abused children in the past, despite no such evidence.	**Checking:** body scans in the genital area to 'test' for arousal when encountered children. **Reassurance seeking:** repeatedly asking friends, family, and spouses for reassurance that they are not a pedophile. **Avoidance:** any situations where children may be present.
Relationship OCD (R-OCD)	Obsessive, disproportionate, and desperate attempts to confirm a romantic partner is the undisputed, 'right' person to be with. Alternatively, it could be Obsessive, disproportionate, and unfounded intrusive thoughts that a partner is cheating, despite no evidence or history of this occurring. Or it could present as intrusive thoughts that a partner will exit the relationship.	**Checking:** checking a partner's mood, obsessively replaying conversations or situation in an attempt to reduce uncertainty. **Reassurance seeking:** repeatedly asking friends, family, and spouses that the person they're with the 'right' person. **Avoidance:** avoiding being emotionally honest for fear of a partner exiting the relationship as a result.

Please note: the particular 'theme' of the OCD or obsessive thoughts is largely irrelevant. Aside from an initial exploration throughout assessment, attempting to understand the theme can become another compulsion. More on this later.

PRESENTATIONS OF OCD II

Sub-type	Description	Common Compulsions
Contamination	Obsessive, disproportionate, and unfounded fears that an individual has come into contact with a contaminated surface or individual, and may transfer this contamination onto others.	**Checking:** repeatedly scanning the body for sensations, smells, and signs of contamination **Reassurance seeking:** asking friends, family, and spouses for reassurance that the individual has not been contaminated, or has not contaminated others. **Avoidance:** any situations where the feared contaminants may be present. **Compulsive handwashing**
Scrupulosity/Religious OCD	Obsessive, disproportionate, and unfounded fears that an individual has incurred the wrath of a significant religious figure, or God, due to intrusive thoughts of having violated religious laws.	**Checking:** compulsively checking religious texts for rules and consequences for violation of said rules. **Reassurance seeking:** an individual asking friends, family, spouses, and religious leaders/community members for reassurance that they are 'good' in accordance with religious laws.
Harm OCD	Intrusive thoughts that an individual has a deep-seated desire to physically harm, or murder, friends, family, or the self, despite no actual desire or intent to do so.	**Checking:** mentally checking for any desire or intent to harm others. **Reassurance seeking:** asking friends, family, and spouses to confirm that the individual is not a sociopath/murderer. **Avoidance:** avoiding objects or situations that prompt intrusive thoughts (i.e., kitchens with knives etc).

Module 1.2: Theory and Treatment

THE COGNITIVE MODEL OF OCD

What is the cognitive model?

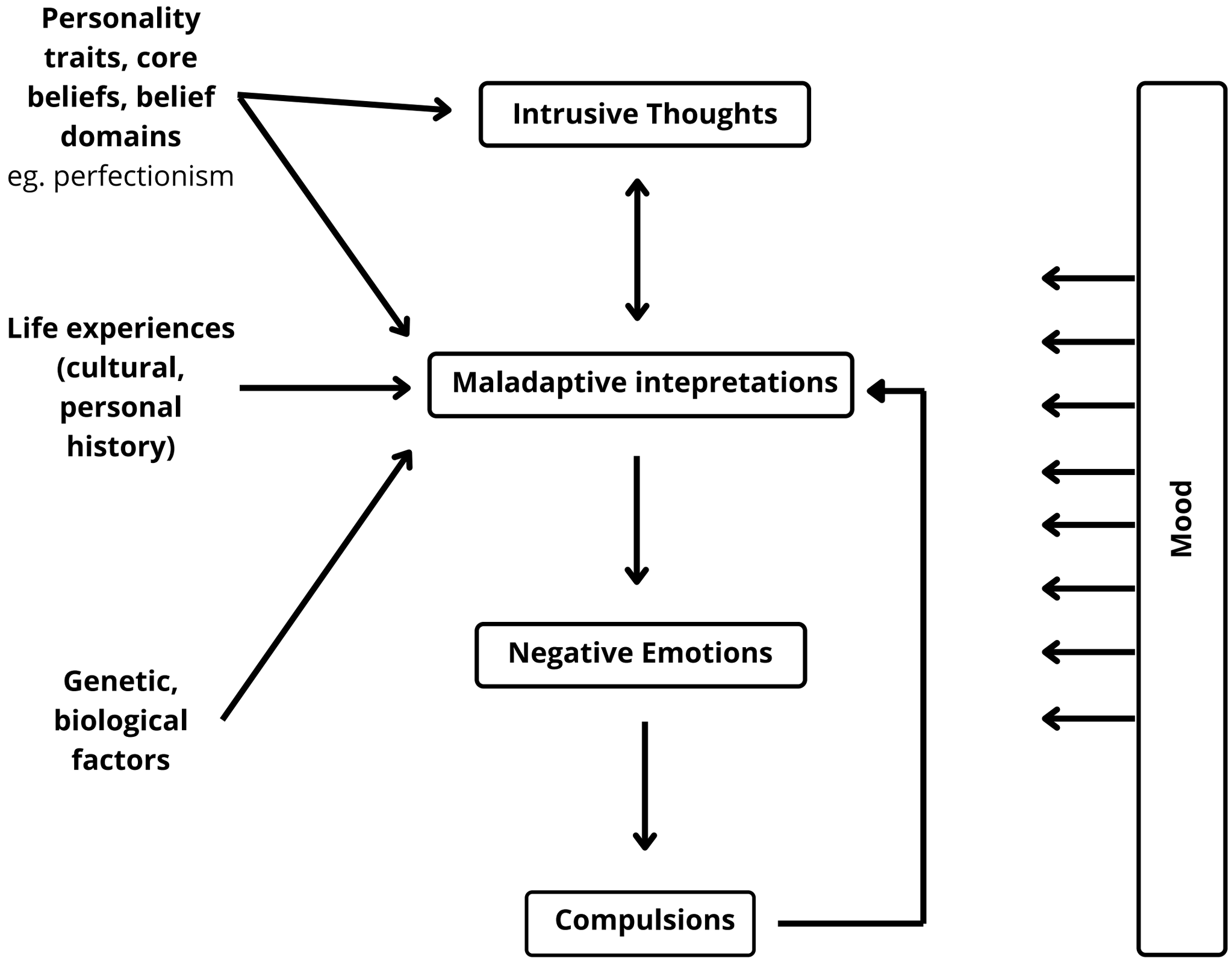
The following theoretical model by Wilhelm and Steketee (2006) provides a succinct overview for how OCD can form, and how symptoms are maintained. Recalling the earlier points on the genetic factors influencing the disorder's development, we can see that these factors combine with life experiences and personality traits/core beliefs to shape our interpretations of the intrusive thoughts we experience. To a greater extent, our interpretations of obsessive thoughts are what begins and sustains our suffering, which is why a substantial component of therapy is centered on changing our responses to thoughts we've labelled, or 'flagged', as dangerous and unacceptable.

Quite often, what OCD 'latches' onto for an obsessional theme is drawn from any prevailing cultural taboos, and the faulty circuitry in the OCD brain remains on high alert for any evidence that we are desirous of engaging in these taboos.

According to the model, personality traits (such as perfectionism) influence the frequency and intensity of intrusive thoughts, and mould our natural inclinations to respond to them in certain ways. After we've labelled a particular thought as harmful, unacceptable, or outright 'wrong', this typically produces a strong negative emotional reaction, or series of negative emotional reactions, including anxiety, dread, sadness, and depression. To regulate these emotions, that is, to attempt to get rid of or reduce them, we then engage in any number of compulsions or safety behaviours, which usually alleviate some immediate distress at the cost of our long-term recovery.

Finally, mood factors have a broad impact on the overall severity of symptoms, in that we're more likely to attribute danger to otherwise neutral thoughts when in an already anxious mood state.

Figure 3. Wilhelm and Sketekee's (2006) Cognitive Model



TREATMENT FOR OCD

The basics

Due to the 'mechanistic' nature of OCD partly arising from a malfunctioning neurological circuit, treatment, therefore, should be systematic and behaviour-based. Contrary to traditional **Cognitive Behaviour Therapy (CBT)** approaches, which seek to challenge irrational thinking by weighing the evidence and plausibility of specific thoughts, successful OCD treatment more closely resembles basic mindfulness practices by way of noticing thoughts, be they rational or irrational, without directly engaging with or attempting to change them.

The initial phase of treatment is about building awareness and insight into the nature of the disorder, which you've largely done by reading up to this point! Next, we'll need a method or process for dealing more healthily with intrusive thoughts, and we'll turn to a type of therapy known as **Acceptance and Commitment Therapy (ACT)** to achieve this.

Finally, we'll encourage our brains to reduce, or 'extinguish', the fear associated with intrusive thoughts by consciously choosing to bring them on them without engaging in compulsions to reduce distress. In doing so via **Exposure and Response Prevention (ERP)**, our brains learn that no harm occurs if we don't engage in safety behaviours, thereby reducing our experience of anxiety and need for compulsions over time.

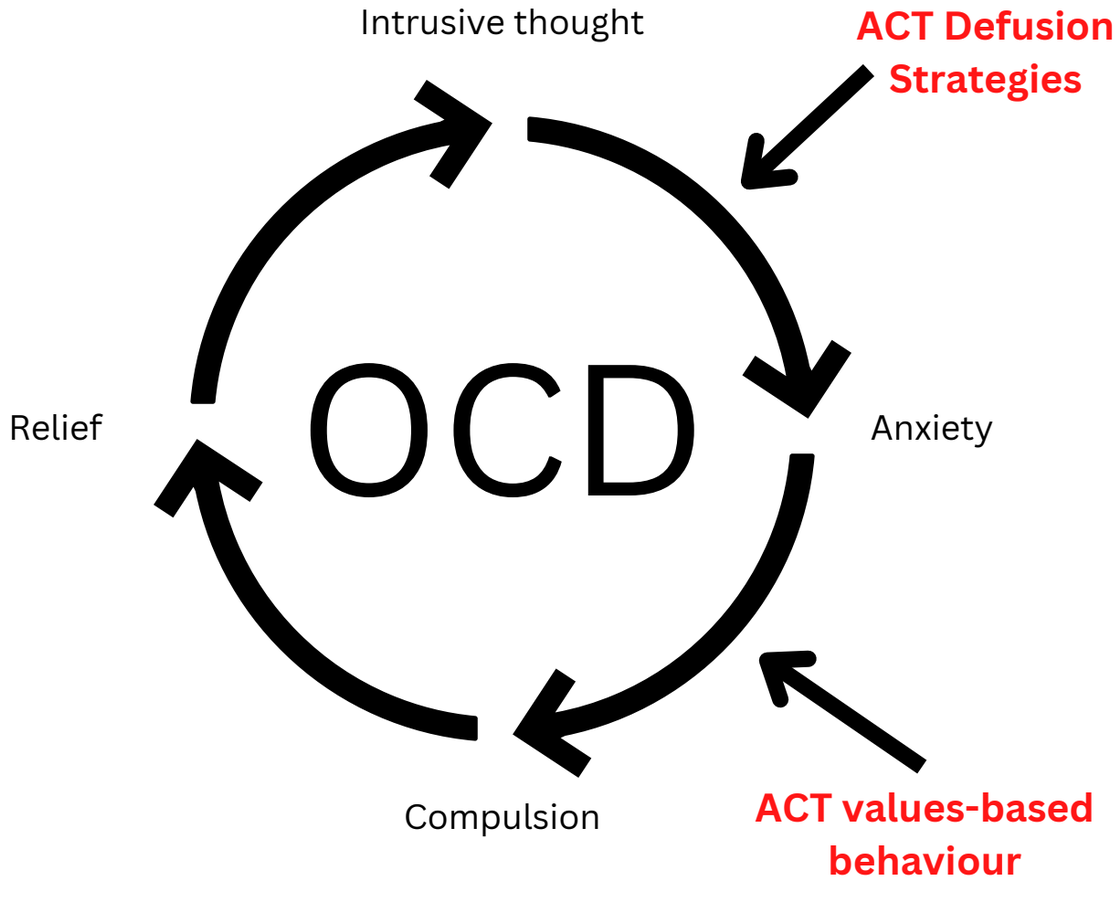
Acceptance and Commitment Therapy (ACT)

ACT builds off the foundational theory of CBT, but has a different underlying philosophy and overall approach to 'treating' or dealing with anxiety, intrusive thoughts, and the resulting compulsions in OCD.

ACT is divided between **acceptance** and **commitment**, with acceptance referring to the whole and total *acceptance* of all material present within our consciousness at a given time, be it thoughts, feelings, impulses or sensations. On the other hand, *commitment* refers to our capacity to make goal-directed and value-based decisions regarding our behaviours, regardless of what other material may be present within our awareness when we decide to take action. Whereas CBT prompts us to directly evaluate each thought, scrutinize it for evidence for and against, and to change it to a more rational or balanced alternative, ACT simply tells us to notice each thought, how it makes us feel and gently return our focus to the present before acting in accordance with our values.

While it might seem strange to do 'nothing' to try and change our way of thinking, ACT does not actually seek to change anything at all aside from our behavioural responses. It simply invites us to leave the mess of irrational thoughts as it is, and to focus elsewhere. It views thoughts as meaningless chatter of the mind, based on outdated perspectives and beliefs, and directs us to 'dis-identify' from our thoughts each time they arise. In doing so repeatedly, we lessen our identification with them, thereby reducing their power over us. In ACT, the real mechanism for change lies in healthy, value-based actions, which bring us closer to the goals and objectives we set for ourselves and our lives. How does this apply to OCD? The diagram below provides an overview:

Figure 3. How ACT interrupts the OCD cycle



As can be seen from **Figure 3.**, the defusion strategies of ACT help to intervene between intrusive thoughts and any subsequent anxiety by inserting a kind-of mental **'circuit-breaker'**. This circuit breaker reminds an individual that an intrusive thought is just that, a *thought*, and in no way holds any value or meaning. While this more adaptive response to intrusive thoughts can feel clunky, strange, and even pointless at first, what one can expect with repeated practice is an increased limiting of the extent to which intrusive thoughts spiral into anxiety-ridden rumination. ***Defusion techniques which can be used in response to intrusive thoughts will be discussed in detail throughout Module 2.***

Secondly, nominating and engaging in values-based behaviours can provide a behavioural circuit breaker to disrupt the impulse, or automatic tendency, to engage in compulsions to reduce anxiety. When combined, these strategies interrupt the often entrenched cycle of **intrusive thought, anxiety, compulsion, and relief.**

Exposure and Response Prevention (ERP)

While ACT is highly effective for reorganising our minds and experiences, particularly in relation to intrusive thoughts, it does not quite provide us with a means to directly confront, or 'face', what we fear. Facing the fears associated with obsessional themes can be a complex undertaking, especially if your obsessional theme is strongly 'imaginative', that is, something not directly accessible in everyday life. Nevertheless, a variety of tools have been developed to account for this, allowing for recovery regardless of the content of intrusive thoughts.

ERP is predicated on the process of **habituation**, which refers to the brains ability to habituate, or reduce its response to, a particular situation, circumstance or thought after repeated exposures or contact. In the context of OCD, habituation occurs when the brain gradually learns to no longer fear an obsessional theme as a result of voluntary contact with it. This contact, referred to as an **exposure**, provides the brain with opportunities to learn that the feared outcomes rarely, if ever, occur. This generally results in the extinguishing of the fear response and a reduction in OCD symptoms.

Exposures are usually co-designed between client and clinician, and should gradually increase in difficulty as therapy progresses until the client has 'faced' their ultimate fear, or what is most anxiety-inducing in relation to their obsessional theme(s). This 'list' of exposures worked through in therapy is referred to as a **fear hierarchy**.

A gentle warning

Exposures are tough. Really tough...

It would be irresponsible to encourage you to actively engage in them without; **a)** the support of a qualified and experienced professional, and **b)** a strong warning about the challenges associated with exposures, but since this is a self-therapy guidebook, we've included some measures to make this process safe and effective - should you choose to engage in it.

Types of exposures

Imaginal

Imaginal exposures are highly effective for individuals with obsessions that are not directly accessible throughout everyday life. For example, if someone experiences existential intrusive thoughts regarding whether or not they actually exist, attempting to design an in-person exposure is nigh on impossible. To remedy this, the imaginal exposure can provide a means for the individual to directly face more philosophical-oriented obsessions. Moreover, if someone is stricken with the obsession that they are not the sexual orientation they've always known themselves to be (sexual orientation OCD), imaginal exposures can serve to challenge intrusive thoughts in a discrete and effective way. It should be stated that some obsessional themes, like sexual orientation OCD, can benefit from a combined imaginal *and* in-person exposure program.

Examples of imaginal exposures include visualising a situation, circumstance or action

that prompts intrusive thoughts while avoiding the use of safety behaviours. Other examples include writing or reading stories relating to obsessional themes, again while refusing to engage in compulsions. These visualisation, writing and reading exposures can be escalated in difficulty in accordance with an individual's motivation and resilience.

In-vivo

In-vivo, or in-person, exposures are exposure exercises in which an individual physically engages with a situation, circumstance or object that prompts intrusive thoughts while abstaining from any safety behaviours used to mitigate anxiety. Examples of in-vivo exposures include steadily increasing contact with a feared location, such as a public toilet, due to obsessive contamination concerns. In this situation, an individual with this particular obsession might first be instructed to look at pictures of public toilets followed by videos of stalls. Next, they would be encouraged to visit one at a distance, while steadily closing the gap between themselves and the cubicle or stall. Eventually someone with this obsession might even use a public toilet as their 'worst-case scenario' or most feared situation, and once they are able to use a public toilet enough times without engaging in compulsions or experiencing overwhelming anxiety, they would have largely 'extinguished' or recovered from their OCD symptoms.

An example fear-hierarchy

For the purpose of this example, let's assume someone is seeking to recover from **Harm-OCD**, which is *the experience of obsessive thoughts relating to physically harming, or murdering, friends, family, or the self, despite no actual desire or intent to do so.* Let's also assume that the intrusive thoughts began after the individual was using a knife to prepare a meal next to a loved one. Below is an example exposure regime, or fear-hierarchy, that could be used throughout recovery:

In-vivo: sit next to loved one with a kitchen knife in full-view.

In-vivo: use a knife to prepare a meal next to a loved one.

Imaginal: imagine using a knife to prepare a meal next to a loved one.

Imaginal: reading stories of true crime murders.

Imaginal: repeatedly reading out *"I am an evil murderer!"*

END OF MODULE 1

In module 1, we developed a thorough understanding of what OCD is, the various explanations for how it manifests, and some of the evidence-based forms of therapy that have demonstrated an ability to reduce symptoms. We also looked at the various 'types' of OCD, including the elusive *Pure-Obsessional* or *Pure-O* subtype, and considered the cognitive model for how symptoms of OCD are maintained.

In module 2, we'll further out understand of Acceptance and Commitment Therapy (ACT) and Exposure and Response Prevention (ERP), and how each of their core principles can be applied practically in service of your recovery.

We'll also be diving into compulsions through an exploration of how and why the develop, and how you can lessen your reliance on them.

Onwards!

Module 2.1: Acceptance and Commitment Therapy (ACT)

ACT FOR OCD

Presence > Premise

While OCD is no longer considered an anxiety disorder per se, the 'language' and main cause for distress as a result of symptoms is, you guessed it, *anxiety*. Because all anxiety disorders, or disorders associated with anxiety, share a common mechanistic function in the brain and body, there tends to be a lot of overlap with respect to the types of therapy used to treat them.

One such, treatment, **Acceptance and Commitment Therapy (ACT)**, has a lot of utility for generalised anxiety disorder, body dysmorphic disorder, health anxiety, social phobia, panic disorder, and, of course, OCD,
If you've skipped ahead to this module and this is your first time encountering ACT as a concept, you may with to read through the outline of this treatment in **Module 1.2** (pg. 9).

In this module we'll explore a core ACT technique, and modify it for use with OCD symptoms. It may seem a little strange or 'basic' at first, but when you understand the mechanisms underpinning it, its inclusion will make a lot more sense!

Defusion

The funfamental way in which ACT can help us to disentangle our identification from our thinking is through defusion. Defusion is the act (pun intended!) of putting distance between our **observing selves** and the thoughts and feelings we encounter minute-by-minute. Perhaps the simplest way to summarise this practice is respond ***to*** the thoughts we encounter, rather than responding ***from*** them.

I totally get that this concept might seem a little strange at first, but if you stop for a moment and start to 'watch' your mind in motion, you'll notice sea of thoughts, impressions, beliefs and perceptions. You can watch them like clouds in the sky, or ducks upon a lake, and yet, they aren't 'you'. For all intents and purposes, you are the watcher observing the activity of your mind.

This is reflected in our neurology, too, with our prefrontal cortex (front of the brain) acting as the seat of this observing self. While various other brain structures relay messages to the observer self by way of thoughts, sensations and emotions, it is the observer self's job to decide which of these messages are valid, and which are unworthy of our precious energy and attention. Obsessive thoughts, while intrusive and uncomfortable, are merely another 'message' sent to us from our brains.

We get into trouble when we start to demonise a particular message as horrible, unacceptable or somehow reflective of our 'true selves', which is how particular thoughts become paired so strongly with overwhelming anxiety. Essentially, our task in therapy is to 'reverse engineer' this process of labelling particular thoughts as unacceptably bad, and to instead treat all thoughts, whether intrusive or not, as neutral fragments of information that we are free to entertain or ignore.

Ever wanted to be a CEO?

To illustrate the point further, imagine you are the CEO (observer self) of a company (your brain). The CEO's job is take in all available information from within the company, and to decide on how the company will spend it's resources, solve problems and maximise profits. Advisors, managers and employees (brain messages) meet regularly with the CEO to report said information, and to offer insights as to the wellbeing of the company. However, the CEO is free to take the offered information or to reject it. The same goes for your brain and the thoughts it likes to gives you. You are free to take on thoughts, feelings and sensations as your identity, or you are free to **practice defusion.**

Again, easier said that done, but, the following exercise will help you on this process towards noticing and accepting the content of your thoughts without accepting them as true.

Defusion technique: Presence versus Premise

Presence: ***"The state or fact of existing, occurring, or being present."***

Premise: ***"The core structural elements of a story."***

In a nutshell, defusion is about accepting the **PRESENCE** of a thought, but not its **PREMISE.** As much as we'd like to, we don't have a lot of direct control over the types of thoughts that enter into consciousness, but we can influence what we experience to a considerable degree (more on this later).

If you've ever struggled with obsessive or intrusive thoughts, you've no doubt attempted to push them away, shut them down, or swap them for another thought, and if any of these techniques were effective, you wouldn't be reading this ebook! The problem with these strategies is that even if they manage to do away with scary thoughts for a moment, we're soon on the hunt, doing all we can to prevent their return. Ever heard the line, *"don't think of pink elephants!"*? Try to not think of the very thoughts you're terrified of popping back into consciousness. It's nigh on impossible.

This is why accepting the presence of a thought is so important. If we extend an 'open door policy' to all thoughts and brain messages, regardless of our feelings about them, they won't need to come knocking with such intensity. They're free to float in and out of consciousness, letting the push and pull of your attention guide them in and out of focus.

To live in fear of a thought is to constantly be on guard, and when they strike while in this mindset, obsessive thoughts can feel 100x more crippling.

So, we've accepted that we're experiencing an

intrusive thought. Now what?

Next, we need to disregard the content, or the premise, of the thought while giving it all the space it requires. We achieve this by:

- **Recognising that this thought is no different to any other.**
- **It only feels real/scary because we've taught our brains to fear it.**
- **Understanding that while we cannot always control the types of thoughts we experience, we can influence the frequency and intensity of certain thoughts by how much attention and focus we give them.**

We can summarise the above into the following response to intrusive thoughts:

"Thanks Brain!"

I warned you that some of these techniques might feel a little strange, but hear me out!

What we're actually doing by thanking our brain is acknowledging that a particular thought has made it's way to our observer self without seeking to explore, analyse, ruminate on or change it. It is precisely these and other follow-up behaviours (compulsions) that feed the cycle of OCD. By accepting the presence, and not the premise, we're free to focus our attention elsewhere, which will likely result in the intrusive thought receding away from awareness once more.

Disclaimer: *this process will take time. We're attempting to change ingrained neural pathways. and like any habit change, this requires time and consistency. You're going to have numerous setbacks, and this is perfectly okay. Once you notice yourself engaging in compulsions, gently remind yourself to accept the presence of the initial thought and leave it at that.*

Let's put this into practice with an example.

Kate has been battling relationship OCD (R-OCD) for quite some time, but she wants to do something about it. She buys this ebook (😉) and learns about the **premise vs. presence defusion technique.** Usually, Kate will experience an intrusive thought about her relationship, which would be something to the effect of:

- **Do I love my boyfriend enough?**
- **How do I know my feelings for him are real?**
- **Is he the right person for me?**

While these questions are perfectly normal in the context of a relationship, Kate is ruthlessly tortured by the uncertainty that never seems to relent, and cannot avoid buying into the 'story' these thoughts present to her. Aside from extreme anxiety, she also feels immense guilt and shame, and is considering leaving her boyfriend despite not wanting to.

Kate begins to practice the ***"Thanks Brain!"*** technique, and at first it feels really strange. There isn't any change to the volume of intrusive thoughts, and she feels like she's wasted her time and money. But soon enough, she finds a rhythm, and starts to accept the thoughts and questions as neutral brain messages. Over time, she's less bothered by the thoughts, and while they still get the better of her occasionally, she is better able to brush them off.

What to do about anxiety

Nothing. No, seriously. Do nothing!

When we're accepting thoughts and no longer giving into compulsions (see next **Module 2.2**), we can expect to experience an increase in overall anxiety. Remember, the reason we do compulsions is to relieve anxiety, so if we're no longer engaging in them then anxiety is likely to rise.

To manage this and reverse the fear associated with a particular intrusive thought, we must become comfortable with discomfort. More specifically, we have to be prepared to feel as anxious as our bodies need to in order to realise that **nothing bad happens when we don't engage in compulsions!**

This concept is referred to as **distress tolerance**, and it's an important skill not only for therapeutic success, but for life, too.

Distress tolerance can be defined by an individual's capacity to tolerate distress, be it psychological, emotional or physical distress. In the context of OCD recovery, distress refers to the emotional distress associated with anxiety. Anxiety in and of itself is not a bad thing - it's adaptive, and can keep you safe when occurring at appropriate times. It can also shelter you within a constricting and suffocating 'comfort zone', taking from you almost everything, including your peace.

Having high distress tolerance does not mean that you do not experience anxiety, but rather when you do, you're able to remain in your observer self, so as to maintain good judgement and decision making skills. It means that your chosen life-path is unaltered by the presence of emotional discomfort. Building distress tolerance throughout OCD recovery is essential, and works much like a muscle in the gym. It takes time to build up, and you always need to apply the right amount of 'stress' to the muscle in order for it to grow. Too much and the muscle won't be able to lift the weight to train. Too little and you won't be stimulating the muscle anywhere near enough. Finding the right amount of discomfort will help you to grow.

So, how do we build distress tolerance? We complete repetitions of facing anxiety directly!

As we know, there are 3 major components to the OCD cycle:

Intrusive thought -> anxiety -> compulsion

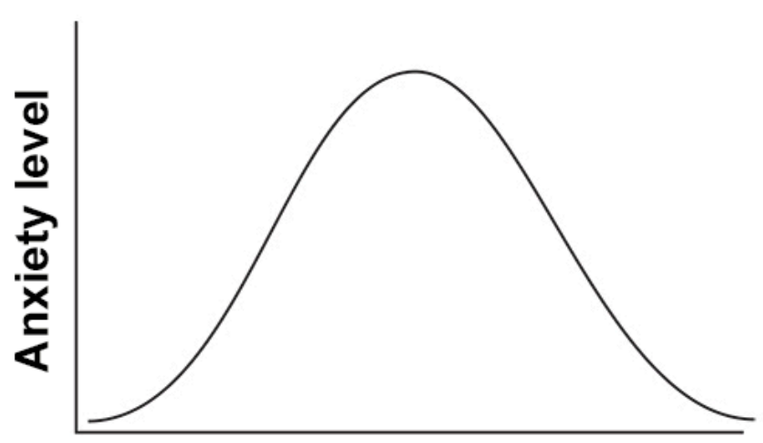
Normally this process happens automatically, but with insight we can begin to slow it down. Once you notice an intrusive thought, see how long you can tolerate the anxiety before jumping into a compulsion. If you can, time this delay using your phone. This will give you a ballpark measure of what you can presently tolerate.

Let's say Geoff's time was 15-seconds. He is surprised at how little tolerance he had for anxiety, but is determined to increase this in service of his recovery. He sets himself a new limit of 20-seconds, and activates it the next time he experiences an intrusive thought. Once he hits the 15-second mark he notices the anxiety increase, but true to his word, he holds off until the timer sounds. At the 20-second mark, he jumps into a compulsion.

Geoff continues this steady increase of time between intrusive thought and compulsion, and after 1 week, he is able to tolerate 2-minutes of anxiety before needing to find relief in ritual. Ideally there would be no compulsions occurring to relieve anxiety, but this is still great progress. Geoff should be really proud of his increased distress tolerance! Perhaps the most important lesson to learn from this exercise is that despite feeling anxious, ***nothing bad happens when a compulsion is not performed!***

Life cycle of anxiety

Once you're able to sit with the anxiety accompanying intrusive/obsessive thoughts for a considerable length of time, it is vital that you remain with the discomfort until it subsides almost completely. Below is a basic diagram of the **life cycle of an anxiety spike** - the very same spike you experience each time you experience an intrusive brain message:



If we jump into a compulsion halfway through this life cycle at the peak of anxiety, we're in effect signaling to our brain that this experience is dangerous and threatening, and that we must remain on constant alert to protect ourselves. Instead, if we wait until the anxiety has subsided back to 'baseline', which is our anxiety level prior to a **spike**, we are showing the brain

that there was no threat, and again, that **nothing bad happens when a compulsion is not performed!**

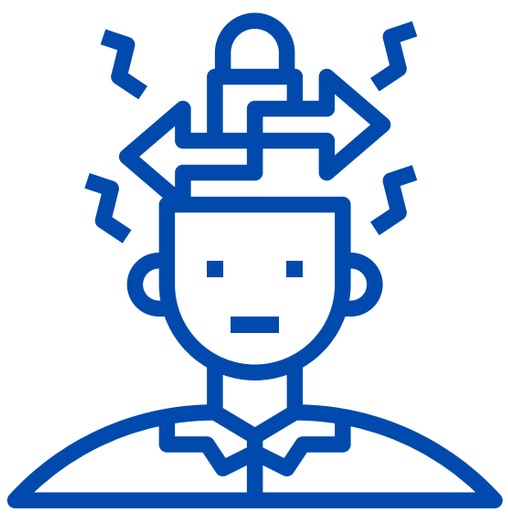
This is how the association between intrusive thoughts and anxiety is slowly eroded over time, eventually leading to intrusive thoughts occurring in the absence of the extreme emotional discomfort you experienced previously. **If that isn't a worthy goal to strive for, then I don't know what is!**

Beware of the backdoor spike!

One thing to be wary of is a **backdoor spike**, which can occur in the initial phases of recovery when an individual is improving. Essentially, the absence of worry and doubt, which is a very strange thing to an OCD brain, can result in a checking frenzy as the brain frantically tries to adjust to the 'new normal'.

In essence, a backdoor spike is the brain's way of attempting to return to 'normalcy', which is the OCD cycle. A really important fact about human nature, and our brains, is that consistency is sought out above all, even above relief from suffering. This is how and why we can get stuck inside all sorts of unhealthy and maladaptive patterns despite 'knowing' or, perhaps on the surface, wanting for things to change. This is core of **resistance**, which is simply the brain's desire to remain as is for the sake of convenience and predictability. Pair this with OCD's refusal to tolerate uncertainty, and you have a recipe for an onslaught of backdoor spikes.

The good news is that you can apply the exact some strategies to a backdoor spike, and remember: backdoor spikes are actually a great indicator of progress! Your brain is 'freaking out' because its not used to feeling unburdened by the OCD cycle. This is cause for celebration.



Summary of the ACT approach

ACT is about accepting the presence of brain messages, be they thoughts, feelings, sensations or impulses, Our choice is not to select which messages we receive, but instead how we respond to the ones we do.

Due to vulnerabilities from a variety of potential sources, those with OCD tend to react strongly to particular themes, none of which are pleasant, and as a result, thoughts related to these themes are deemed inconceivable, unacceptable and dangerous. The worst fear for the OCD sufferer is that the thoughts are in someway indicative of their true desires, or that the catastrophic content of obsessions is going to occur.

To remedy this:

- **We first remain open and accepting of all brain messages, including obsessive thoughts. Keep in mind the *'open door policy'*!**
- **Next, we practice building up our distress tolerance by increasing the spaces of time between the anxiety following an intrusive thought and relief-seeking compulsions.**
- **Once we've built up enough distress tolerance, we should strive to sit with the anxiety following an intrusive thought until that anxiety returns to baseline - aka, the level of anxiety prior to the intrusive thought or spike. This will erode the fear associated with obsessional themes, and eliminate the need for compulsions.**

After successfully raising your distress tolerance, and sitting through anxiety spikes associated with intrusive thoughts until the anxiety subsides, hopefully you'll come to appreciate the deep realisation that compulsions are the enemy, not the solution.

Alternatives to compulsions

ACT provides us with both the 'open door policy' for all brain messages by way of *acceptance*, and also a guide for more healthy and adaptive behaviour by way of *commitment* to **values-based behaviour**. In the context of OCD, values-based behaviour includes any healthy behavioural choice that is in line with an individual's values, or their life goals.

For example, Ella has worked really hard to increase her distress tolerance and to be more accepting of all thoughts, even the ones she'd prefer not to have. While this has been largely successful, she's at a loss as to what to do with herself now that she can longer engage in compulsions to the same degree. As a result of the exercise outlined below, she identifies her values to be: **kindness, being organised, and gratitude**. She then identifies a series of activities related to each of these values. As an example, gratitude journaling is a behavioural extension of her value of gratitude.

Moreover, she wishes to enroll into university to study nursing, but never felt she was able to commit to the workload due to OCD compulsions taking up the bulk of her days. Now that she has more time, she's revisited this life goal, and has decided to apply for the course.

Compulsions can completely overtake a person's life when in the throes of OCD, and they can leave a gaping hole once they're lessened or stopped. To avoid being 'sucked back in' to the cycle of OCD, it is vital that we occupy our day with healthy values/goal oriented activities. **After all, Idle hands are the OCD devil's playthings!**

To achieve this, we need to identify our personal values using the following exercises.

Values

An understanding of your core values will provide you with a 'road-map' of how to rehabilitate your life while in recovery from OCD and beyond. Values are our navigators through the unpredictability and challenges of life, and can be used to orient ourselves towards a consistent life path, even when experiencing great personal difficulty.

Let's begin.

Values exercise 1

To determine your values, ask yourself the following questions:

- **When were you happiest?**
  - What were you doing?
  - Who were you with?
  - What was it about the situation that made you happy?

- **When were you most proud of yourself?**
  - Why were you proud of yourself?
  - Did anyone else share in your pride? If so, who?
  - What other factors contributed to your sense of pride?

- **When were you most fulfilled or satisfied?**
  - What need or desire was fulfilled?
  - What was/were the activity or activities?
  - What other factors contributed to your satisfaction?

After answering the 3 questions, attempt to match your responses with a corresponding value in the values list available on page 14. If you find yourself with more than 4 values, see if you can combine values into a representative value. For example, you may combine 'helping society', **'justice'** and **'charity'** into a grouping value like **'compassion'.**

Values exercise 2

Think about a real-life person or fictional character that you admire. What is it about them specifically that invokes your admiration? Note down their virtues and attributes, and if

you are having difficulty doing so, confer to the values list on page 14 to decide which values apply to them. And remember; try to keep it between 2 and 4 values for the sake of conciseness.

On the following page you will see a list of values to help you identify which are most applicable to your and your situation. On this page below is a **values and life goals activities table**, which is fairly self explanatory. Once you've identified your values and life goals, consider a range of activities that correspond to each of them that you can engage in when emerging from the haze of life within the cycle of OCD.

Values and life goals activity table

Values	Activities
eg. Self-reliance	Budget my finances for the week and determine a saving plan
Life goals	**Activities**
eg. Finding part-time work	Update my resume and bookmark some jobs online

Values list

Accountability
Accuracy
Achievement
Adventurousness
Altruism
Ambition
Assertiveness
Balance
Being the best
Belonging
Boldness
Calmness
Carefulness
Challenge
Cheerfulness
Clear-mindedness
Commitment
Community
Compassion
Competitiveness
Consistency
Contentment
Continuous Improvement
Contribution
Control
Cooperation
Correctness
Courtesy
Creativity
Curiosity
Decisiveness
Democraticness
Dependability
Determination
Devoutness
Diligence
Discipline
Discretion
Diversity
Dynamism
Economy
Effectiveness
Efficiency
Elegance
Empathy
Enjoyment
Enthusiasm
Equality Excellence
Excitement
Expertise
Exploration
Expressiveness
Fairness
Faith
Family-orientedness
Fidelity
Fitness
Fluency
Focus
Freedom
Fun
Generosity
Goodness
Grace
Growth
Happiness
Hard Work
Health
Helping Society
Holiness
Honesty
Honor
Humility
Independence
Ingenuity
Inner Harmony
Inquisitiveness
Insightfulness
Intelligence
Intellectual Status
Intuition
Joy
Justice
Leadership
Legacy
Love
Loyalty
Making a difference
Mastery
Merit
Obedience
Openness
Order
Originality
Patriotism Perfection
Piety
Positivity
Practicality
Preparedness
Professionalism
Prudence
Quality-orientation
Reliability
Resourcefulness
Restraint
Results-oriented
Rigor
Security
Self-actualization
Self-control
Selflessness
Self-reliance
Sensitivity
Serenity
Service
Shrewdness
Simplicity
Soundness
Speed
Spontaneity
Stability
Strategic
Strength
Structure
Success
Support
Teamwork
Temperance
Thankfulness
Thoroughness
Thoughtfulness
Timeliness
Tolerance
Traditionalism
Trustworthiness
Truth-seeking
Understanding
Uniqueness
Unity
Usefulness
Vision
Vitality

Module 2.2: Compulsions

THE ANATOMY OF A COMPULSION

How it starts

As we've learned, compulsions are the 'fuel' maintaining the cycle of OCD. If you can can imagine the OCD cycle as a snarling beast, then our ritualised behaviours aimed at reducing our emotional distress are it's sustenance. Without it, it starves and eventually dies. This is why so much of the therapeutic process for OCD is about reducing compulsions, and ultimately learning to better tolerate the anxiety that undoubtedly arises when we stop running from it.

Again, this is a tall order, which is why this chapter will focus on how compulsions form, and what you can do to steadily reduce your reliance on them.

So, let's get started!

Compulsions are unhelpful for a number of reasons, but perhaps the most obvious is that each time you engage in one, you're robbing your brain of the opportunity to realise that, again, **nothing bad happens when a compulsion is not performed!** (starting to notice a pattern here?)

Brains learn by experience, and, interestingly, this experience does not have to be derived from physical engagement with our environment. So, if we experience anxiety following an intrusive thought and we jump immediately into a compulsion, the brain's 'experience' in that moment is ***the intrusive thought is dangerous and says something horrible about me***, when, in actual fact, this is simply not the case. Recovery from OCD is allowing your brain the healthy experience of learning

that ***not performing compulsions does not lead to harm.***

Here's an example of how compulsions can form:

Jason is sitting with his family watching television, and a ripped male actor enters the scene of the program they're viewing. Suddenly, and without reason, Jason experiences the thought ***"what if I found that sexually arousing?"*** Jason, who has known himself to be straight his whole life, is suddenly stricken with the fear that he's gay, and is flooded with thoughts of having to break the news to his family, and leave his long-term girlfriend despite loving her all the same. While it is perfectly understandable for us to review questions about our identity on occasion, the urgency and subsequent repetitive nature of these and similar intrusive thoughts would suggest that Jason has unwittingly entered the cycle of OCD:

- He begins to 'check' his body for arousal every time he sees a conventionally attractive male, and despite zero indication of attraction, his brain does not relent. Sometimes he feels immediate relief after a check, which is why his brain has learned to prompt the behaviour each time an intrusive thought related to his sexuality pops up.

- Further still, Jason has begun avoiding his male friends, as the past few times he's been near them his intrusive thoughts worsened in intensity and frequency. Although Jason is saddened by this, he places greater value on the feeling of relief he experiences when cancelling plans.

- Finally, Jason has tried to seek reassurance about his previously unquestioned sexual identity by asking his girlfriend repeatedly if she things he's gay. While their sex life continues unaffected and Jason is just as satisfied with their intimacy, her saying something to the effect of "of course not!" provides a gratifying sense of temporary relief.

As you can see in the example of Jason, the genesis of a compulsion is quite simple:

Whatever we begin to do around the time we first experience intrusive thoughts tends to become our entrenched 'go to' relief rituals.

Luckily for Jason, he went on to realise that these questions were not genuine questions about his identity, but rather his brain's mispairing of these questions with profound anxiety, which is why the urgency felt so real.

Through therapy, Jason was able to learn to accept the presence of the intrusive thoughts without resorting to his outdated and unhelpful compulsions. As his capacity to tolerate anxiety and distress increased, the intensity and frequency of intrusive thoughts decreased proportionately.

Here are some important questions to ask yourself:

- **What felt natural or normal to do as soon as I started to experience intrusive thoughts?**

- **What value did these behaviours hold for me?**

- **Do I still believe these behaviours hold the same value for me now?**

- **What do I believe would happen if I were to stop these behaviours?**

Why can't I stop!?

Firstly, you absolutely can stop - ***this I can confidently assure you of***. As we've outlined in previous modules, you brain has paired the compulsion, regardless of what kind you use, with **relief**. Relief from pain, be it physical or emotional pain, is a powerful reinforcer for behaviour, which is, in some ways, how addiction can start. You can think of compulsions as an addiction of sorts, as the rush of relief floods your nervous system with a wave of the reward-hormone dopamine. This same hormone is released when we anticipate a reward, consume drugs or alcohol, and achieve a goal or objective.

Naturally, our brains like to feel good, so the next time it kicks off the OCD cycle with an intrusive brain message, you can almost view as your brain's way of attempting to get its next 'hit', so to speak. Pair the metaphor for addiction with the fear of anxiety, and you have a pretty good explanation for why you feel you can't stop.

Ranking your compulsions

In the same way you were encouraged to rank your fears in relation to specific obsessional themes from least to most anxiety-inducing, a great exercise is ranking your compulsions from least to most difficult to eliminate.

Let's put this in the context of an example:

Rosey has identified that she struggles with obsessive thoughts, and although she hasn't received a formal diagnosis of OCD, she wishes to follow the advice in this eBook. Her particular theme is the obsession that she'll contract AIDS from a used needle on the ground. As a result, she avoids walking on footpaths almost completely and, as you can imagine, this greatly interferes with daily life. Her compulsions include avoiding footpaths, researching the likelihood of contracting AIDS, and checking her body for symptoms.

1) Avoidance of walking on footpaths
2) Ruminating over the obsessional content
3) Checking for symptoms of AIDS

Ranking Rosey's compulsions

Rosey decides that her avoidance of walking on footpaths for fear of being pricked by a needle is the hardest compulsion of hers to stop, and that checking for AIDS symptoms would be the easiest.

As a result, she decides to reduce her checking first. To achieve this, she records the number of times she catches herself checking her body for symptoms on a given day, and decides to reduce the amount of times she's allowed to check by 1 for the next few days.

She scans the compulsion counter resource (pg.) and inputs the number of times she's permitted to perform the compulsion for the next day. When she has used up all her tokens, she commits to feeling the anxiety associated with the intrusive thought that she's experiencing bodily symptoms of AIDS, and to deny herself the urge to give into the compulsion. She instead sits with the anxiety and waits for it to peak and then trough (see the **anxiety curve** on page 10). After a few days of repeating this process, she notices that there is substantially less anxiety when she's out of compulsion tokens as compared to previous days.

She steps up the challenge and reduces her available compulsion tokens further by one and repeats the process until she is no longer using any compulsion tokens in the face of intrusive thoughts. Eventually, she does the same for her other compulsions, although she discovers she needs to go much slower, as these compulsions are a little more entrenched.

Compulsion reduction strategy: compulsion tokens

Because recovery from OCD is a gradual and challenging process, we need to progress at a steady, but slow rate. Utilising compulsion tokens is a very effective way of gradually reducing reliance on safety behaviours and avoidance of anxiety simultaneously. The beauty of this approach is that you are in control of how fast or slow you travel.

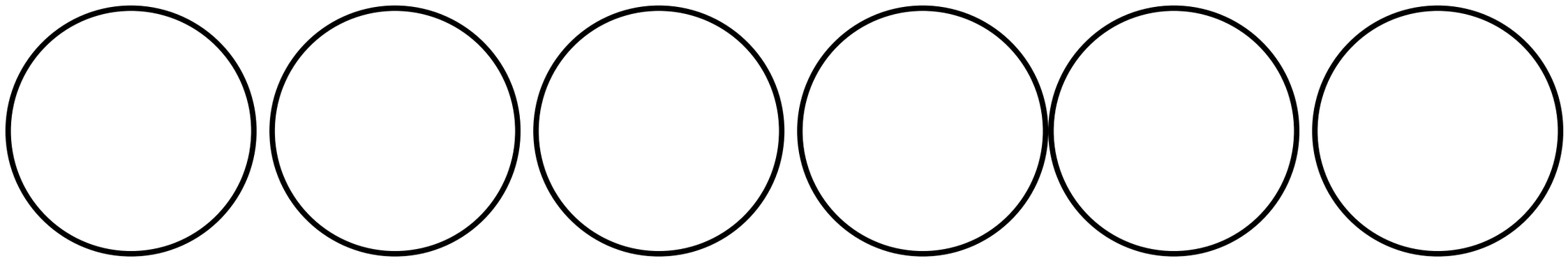
Compulsion reduction strategy: compulsion tokens

Blank template:

Day:

Compulsion:

Compulsion tokens

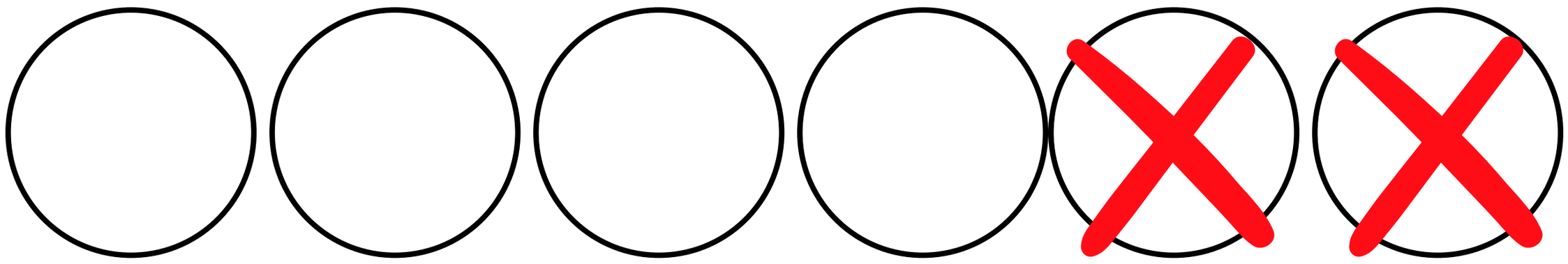


A worked example:

Day: Wednesday

Compulsion: Avoidance of footpaths

Compulsion tokens



In the above worked example, Rosey has progressed to only allowing herself 4 compulsion tokens per day, which is why she's crossed out the 2 tokens on the righthand side. As she progresses further, she will add crosses to reduce her available tokens until she is not relying on any to engage the anxiety associated with intrusive thoughts.

END OF MODULE 2

Module 2 began with a deep-dive into Acceptance and Commitment Therapy (ACT), and outlined some of its core techniques, including the acceptance of mental material (regardless of how distressing), defusion and utilising our values to guide us towards our chosen goals and life path.

Following this, we looked into the anatomy of a compulsion, and how to break the cycle of reliance upon them. To achieve this, a simple ‘compulsion token’ technique was provided to steadily reduce the frequency with which we engage in safety behaviours.

CONGRATULATIONS!

You have made to the end of the Self-Therapy Guidebook for OCD! This is no small feat, as challenging our habits, some of which have been lifelong, is an arduous and exhausting process, but one that is ultimately rewarding and inspiring. While this guidebook was intended to be read from start to finish as a simulation of a course of therapy administered by a professional, it is highly recommended that you return to any section of the content as needed to maintain upkeep of your emerging recovery skillset.

As a final note, any process of personal change is hard, and requires reserves of willpower, disciplined action, and a clear goal or 'why' for undertaking it. Mental health, like anything else, is largely a composite of healthy micro-behaviours conspiring to improve your overall quality of life, so take it slow, and steadily build the behaviours necessary to facilitate this process.

Once again, congratulations for taking steps forward to improve your life, and we wish you the very best on the road to recovery from OCD.

The Greylight Psychology team